Learning Outcomes.

1. To understand the purpose of an inquest and the difference between a pre-inquest review hearing and an inquest.
2. To use the framework and tips provided to write a clear and comprehensive report.
3. Recognize the difference between a witness of fact and an interested person.
4. Be able to undertake the necessary preparation to be able to negotiate the process and reduce the risk of a negative outcome, such as criticism by the coroner.

Section 1. The purpose of an inquest.

In this section, we will look at:

- A. Purpose of an inquest, including notification and the difference between a pre-inquest review and an inquest.

"It should not be forgotten that an inquest is a fact-finding exercise and not a method of apportioning guilt." (Lord Lane LCJ, 1982).

1.1: The coroner's court

Whereas criminal and civil courts are adversarial in England and Wales, the coroner's court is inquisitorial. The coroner asks questions of the witnesses to establish the facts and understand decisions, actions and omissions that may have contributed to or prevented death. It is a formal court proceeding that is held in public.

1.2: The purpose of an inquest

A coroner must investigate deaths that are violent, unnatural or unexplained and those that occur in state detention (Section 1, CJA 2009).

There are three discrete phases in the coronial process (most notifications to the coroner do not get beyond the first phase):

(1) preliminary inquiries (before formally opening an investigation);
(2) an investigation (with or without inquest); and
(3) an inquest.

A preliminary inquiry or investigation, including a post-mortem, may reveal a natural cause of death, not occurring in state detention (such as being sectioned) and if this is the case, the coroner will not then hold an inquest.

1.3: The scope of the inquest

Ultimately, a Coroner's inquest must answer four questions:

- Who the deceased was (their identity),
- When and where they came by their death
- How (by what means).

The death of a psychiatric patient detained under the Mental Health Act will engage Article 2 (the 'right to life') of the European Convention on Human Rights (EHCR), which means that the inquest must be conducted with the 'how' the deceased came by their death broadened out into 'how and in what circumstances' they came by their death (R (Middleton) v HM Coroner for Western Somerset [2004]), resulting in a more in-depth inquiry by the coroner. The law states a jury must be called in this circumstance if the death is unnatural.

1.3 What does the jury do?

The role of a jury in an inquest is different from that of a criminal case and only about 1% of inquests will have a jury (Ministry of Justice, 2024).

An inquest into a death is held without a jury unless the senior coroner suspects it falls into one of the following categories;

1. The deceased died while in custody or otherwise in state detention, and that either:
   (i) the death was violent or unnatural; or
   (ii) the cause of death is unknown.
2. The death resulted from an act or omission of a police officer or a member of a service police force.
3. The death was due to a notifiable accident, poisoning or disease (CJA, 2009, section 7).

In every jury inquest the coroner decides matters of law and procedure and the jury decides the facts of the case and comes to a conclusion which must include the legal 'determination' and 'findings', including the cause of death.

Like the coroner, the jury cannot blame someone for the death. If there is any blame, this can only be established by other legal proceedings in the civil or criminal courts. However, the jury can state facts which make it clear that the death was caused by a specific failure of some sort or by neglect.

Juries are called in the same way as juries in other courts (from the Electoral Register) and consist of between seven and eleven members.

1.4 Notification of involvement in an inquest

In most instances, the doctor will hear from the family or other healthcare professionals that a patient has died. However, on occasion they will find this information out directly from the coroner either in writing or through a phone call.

If the coroner has decided an inquest is necessary to investigate the death, one must be opened as soon as practicable after a reportable death, and the process should, where possible, be concluded within six months.

In complex cases the coroner may hold a Pre-Inquest Review Hearing (PIRH) with interested persons, to decide on the scope of the investigation, identify witnesses, and to plan the inquest date and duration.

As soon as you are aware of an inquest it is sensible to contact your Medical Defence Organisation (MDO) as they can then liaise with the coroner on your behalf if they believe that this is necessary

1.5 A Pre-Inquest Review Hearing (PIRH) and how it differs to an inquest.

In complex cases the coroner may hold a Pre-Inquest Review Hearing (PIRH) with interested persons, to decide on the scope of the investigation, identify witnesses, and to plan the inquest date and duration. The purpose is to assist in the management of the inquest itself. This is particularly useful in complex cases. The coroner would normally alert you to a PIRH, this can sometimes be at very short notice.

At the PIRH, the Coroner will want to determine:

• Identity of Interested Persons

• Scope of the Inquest

• Issues to consider

• Whether the Right to Life, Article 2 of the European Convention of Human Rights (ECHR) is engaged2

• Whether a Jury is required

• Witnesses (Rule 23 or live)

• Length of hearing and set hearing dates

• Disclosure.

1.6 Rule 23 (R23)

Something that is determined before the inquest or at the PIRH is whether the witnesses will give their evidence live or whether their statements will

be merely read, pursuant to Rule 23
https://www.legislation.gov.uk/uksi/2013/1616/article/23

It may not be possible for a doctor to give evidence in person or online due to clinical commitments and if the evidence is unlikely to be disputed, written evidence may be acceptable. This would need to be agreed in advance of the inquest and your trust or MDO can make this request on your behalf.

Section Recap

1. Coroner's inquest must answer four questions: who died, where, when, how, or in what circumstances the deceased came by their death. TRUE

2. In complex cases the coroner may hold a Pre-Inquest Review Hearing (PIRH) with interested persons, to decide on the scope of the investigation, identify witnesses, and to plan the inquest date and duration. TRUE

3. At the PIRH, if the case is complex and requires this, the coroner will want to determine identity of Interested Persons, scope of the inquest, issues to consider, whether the Right to Life, Article 2 of the European Convention of Human Rights (ECHR) is engaged, whether a Jury is required, witnesses (Rule 23 or live), length of hearing and set hearing dates, disclosure. TRUE

Section Summary

In this section, we covered the following:

- Purpose of the inquest – fact-finding exercise, not for apportioning blame.

- Notification may be by the family or a colleague but can be directly from the coroner. It is important to involve your trust legal team, if applicable and your medical defence organisation at an early stage as they can support and liaise with the coroner as needed.

- Purpose of a pre-inquest hearing – to aid planning of complex cases.

- When a jury is required.

- Rule 23 (written evidence).

Section 2 Request for Statement

In this section, we will look at:

- **Reporting writing for an inquest**
- **Dos and Don'ts of statements**

2.1 A request for a statement (sometimes referred to as a 'report').

If you have been involved in the care of a patient whose death is referred to the coroner you may be asked to provide a statement outlining your involvement.

The production of a comprehensive, clear but preferably concise report can have a number of positive outcomes:

• Aid the Coroner in their understanding of events.

• Give closure to the family by answering questions they may have.

• Provide some catharsis for the clinician, it is very rare for a clinician to not have doubts about their practise, or if issues/errors are identified, it allows time to remediate and address these issues prior to the inquest.

• A well-prepared report is excellent preparation for giving evidence.

Reflection – what information do you think should be included in a statement for the coroner?

The report should be based on the medical records, your own recollection and your usual practice. What to include:

• Personal details – your qualifications, number of years working, relevant clinical experience and background
• Who has requested the report and for what purpose – what you have been asked to include in the report
• Details of other healthcare professionals involved
• Patient details
• Summary of patient's medical issues and medication history
• Chronology of important events, increasing in level of detail up until the patient's death, with the most detail in relation to the last consultation
• An offer to answer any further questions that may arise and condolences to the family
• The report should be clearly dated and must be signed by you.

2.2 Dos and don'ts

Do…

• Write your report honestly; don't be influenced by others

• Write it as soon as possible, while the incident is still fresh in your mind

• Only include details of events that you personally were involved in, unless attributed to others for example "Mr X was seen by Dr Y on...., the medical notes indicate..."

• Only include relevant facts; your opinion is only necessary if specifically asked for

Do not...

• Exceed your level of competence

• Don't comment on behalf of others – but you can say "Dr X said...."

• Deliberately conceal anything – this will cast doubts on your integrity and will make subsequent comments less credible.

2.3 Statement writing tips

• Write in the first-person singular – "I did this..."

• Address the report to an intelligent lay person; avoid jargon and abbreviations

• Bear in mind that the patient or their relatives are likely to see the report; avoid any pejorative, humorous or unnecessary subjective remarks

• Organise the report chronologically – give actual dates, and use either a 24-hour clock to give times, or state whether you are referring to am or pm

• Give each incident, or event, a separate paragraph or section

• Check spelling, punctuation and grammar before submitting, even minor errors can be viewed negatively by a grieving family

• Your report should be typed, signed and dated

• Keep a copy of the report in your notes and a note of how, when and to whom you submitted it.

If you are asked to change the report, you should think very carefully about the event before doing this, and only make changes if a factual mistake needs to be rectified.

Making a supplementary statement

Sometimes it is necessary to make a supplementary report to deal with issues that come to light after you have written your original report. Before doing this, make sure that you review your report, the medical records and any new documentation.

Section Recap

1. **Taking time to produce a clear and comprehensive statement can provide catharsis for the clinician and act as excellent preparation for giving evidence. TRUE**

2. **Your statement should be typed, signed and dated. TRUE**

3. **It is important to submit your statement as quickly as possible. FALSE**

   You must ensure that your statement is factually accurate, this will require you to locate and review the medical records. You may also wish to seek a review of your statement by your indemnifier or trust legal team. These steps will take time.

Section Summary

In this section, we covered the following:

- **The production of a clear, comprehensive statement, the format and what the statement should include.**
- **Dos and Don'ts of report writing.**
- **Report writing tips.**

Section 3. Attending an inquest as a witness

Healthcare professionals often play an important role in proceedings. You can be called to attend an inquest as a witness in different ways:

- **as a 'witness of fact' with your statement forming the basis of your oral evidence**

- **as an interested person: if you are granted this status, it indicates that the coroner believes you to be more centrally involved in the circumstances leading to the death**
- **as an expert witness.**

It is crucial that a clinician being asked to make a statement in a coroner's court understands their legal status and why they have been asked.

In this section, we will look at:

- **GMC guidance**
- **Differing types of witness**

3.2 Good Medical Practice (2024).

The GMC says you must be honest and trustworthy when giving evidence. You must make sure that any evidence you give or documents you write, or sign, are not false or misleading. The GMC expects doctors to adopt this approach when acting as a professional witness, as an expert witness and when giving evidence in a non-professional capacity. You should also recognise and work within the limits of your competence.

Paragraph 89:

You must make sure any information you communicate as a medical professional is accurate, not false or misleading. This means:

a. you must take reasonable steps to check the information is accurate
b. you must not deliberately leave out relevant information
c. you must not minimise or trivialise risks of harm
d. you must not present opinion as established fact.

3.3 Witnesses

In general terms. if you are called as a witness, your role is to provide impartial evidence to help the court reach its decision. There are different types of medical witnesses:

Professional witness – you are being called to supply factual information obtained in your capacity as the treating doctor in a particular case; it is important to stick to the facts, and not to stray into providing opinion, beyond the scope of your expertise.

Expert witness – you are being called to give an independent opinion on the facts of a case, based on your own specialist knowledge and experience, and usually without any personal involvement with treatment in the case.

You can be called to attend an inquest hearing as a witness in two ways:

As a “Witness of Fact”: If you are called as a witness of fact, it would usually indicate that the coroner believes your involvement in the case to be peripheral. Your statement will form the basis of your oral evidence and you may be asked questions in order to clarify certain aspects.

• As an “Interested Person” (IP): If you are called as an IP it would indicate that the coroner believes you to be more centrally involved in the circumstances leading to death. If you are granted IP status you are entitled to have legal representation; receive disclosure of the documents that the coroner intends to rely upon at the inquest; and ask questions of other witnesses.

For these reasons, and because there is a higher risk of criticism, it is important to know if you have been granted IP status at the inquest. There are a number of statutory grounds under which a person can be given IP status. They are set out at section 47(2) a-m of the Coroner’s and Justice Act 2009.

Doctors and medical professionals are usually recognised under section 47(2)(f):

“A person who may by any act or omission have caused or contributed to the death of the deceased, or whose employee or agent may have done so”.

Also, the coroner has a “catch all” discretion under section 47(2)(m) to grant IP status to anyone who is deemed to have “sufficient interest”.

Witness evidence is given under oath, which means that you are under a legal obligation to tell the truth at an inquest.

3.4 Going to the coroner’s court
You will usually be put on notice that your attendance is required and asked beforehand for dates that are convenient to you. If you are served with a witness summons, you must attend at the specified time and for a set duration. You must comply with a witness summons, otherwise you risk being found in contempt of court – this is a criminal offence.
If you believe that you have a legitimate reason for being unable to attend, you should seek advice from your indemnifier.

3.5 Preparation for going to the coroner’s court
Beforehand:

- Read through your report

- Review the medical records

- Be clear who has called you to attend
- Find out where the court is and how long it will take you to get there
- Find out how long you will be needed for
- Make sure the medical records will be available at the court
- Make sure you have adequate cover arrangements in place.

3.6 On the day

In most courts the coroner sits at the head of the chamber, with the witness box to one side. The advocates' bench is in front of the coroner, and behind that is general seating. Inquests are held in an open (public) forum and some will generate media attention so reporters may be present. Additionally, all inquests are now audio-recorded.

• Be prepared - reading statements that you have submitted, and any other relevant documents will refresh your memory of events. You may take case documents into the court room and refer to these while in the witness box. Consider marking or highlighting important sections in advance for ease of reference on the day.
• Dress smartly. This is a formal occasion and a suit or other business attire is considered appropriate.
• Arrive on time. Register your arrival with the coroner's officer or clerk. You will be shown into the Courtroom and be seated ready to be called to take the witness box.
• Switch your mobile phone off!
• Be polite and remember it is fine to give your condolence to the family if you see them at court.
• When called, you will be asked to 'swear in' by reading an oath on a holy book, or a non-denominational statement of truth. After this point, any failure to tell the truth would amount to perjury.
• Each witness will be questioned by the coroner and then by interested persons, including potentially the family.
• Direct your answers to the coroner, whom you should address as 'Sir' or 'Madam'.
• Answer the question you have been asked, not the one you wanted to be asked. If you are unsure of the question, seek clarification.
• Keep your answers brief and factual.
• Use non-technical language whenever possible. Ideally you should answer in lay terms.
• Keep calm. The tendency is to speed up when nervous, but make sure you speak clearly and slowly.
• Remember that media may be in attendance, dependent on the level of press interest in the case. Statements made during an inquest may be

covered by media outlets if the case is open for press to attend. If asked for additional statements by media before, during or after the inquest, contact your MDO before making any comment, to get media advice and support.

Avoid…

• Discussing the facts of the case outside the courtroom as this could be seen as interfering with the inquest process.
• The temptation to fill gaps in proceedings by speaking. The coroner may be making notes from the evidence or preparing their next question.
• Straying into the remit of an expert unless you are qualified to give such an opinion. Remember that a witness of fact should confine evidence to facts within their direct experience.
• Leaving court before the coroner releases you.

3.7 Tips on giving a successful performance in the witness box:

- Remember that you are impartial – your duty is not to one side or the other.

- Speak clearly, using short sentences – try not to over-elaborate and explain any technical terms you may have to use.

- You are giving evidence to the judge/coroner/ chairman, so ensure that you face them when answering a question.

- Listen carefully to each question. Make every answer open, honest and fair.

- If you don't know the answer, or understand the question, say so.

- Don't lose your patience with the opposing counsel. Lawyers are working on behalf of their clients and disparaging comments can be a deliberate tactic – the best witnesses are those that remain neutral and focused.

- You can appeal to the coroner if you feel that a question is improper, or if you would like to expand on your answer. Remember to take as much time as you need for each answer. A conscientious witness will pause for as long as necessary before speaking to ensure that they are giving evidence that really is "the truth, the whole truth and nothing but the truth".

Section Recap

1. You should address the coroner as 'Sir' or 'Madam'. TRUE

2. **Leave the court when you have finished your evidence. FALSE**
   The coroner releases you and then you are able to leave the court. If unsure clarify if you have been released before leaving.

3. When called to give evidence, you will be asked to 'swear in' by reading an oath on a holy book, or a non-denominational statement of truth. TRUE

Section Summary
In this section, we covered the following:

1. **When giving evidence you must be honest and trustworthy, take reasonable steps to check the information you provide and not deliberately leave out any relevant information.**
2. **You must establish with the coroner if you are a witness of fact or an interested person prior to attending the inquest, as this determines if you can have disclosure of all evidence to aid you with giving your evidence.**
3. **Success in the witness box is aided by careful preparation.**

Section 4 Post inquest
In this section, we will look at:

- **What happens after the inquest.**
- **What is a regulation 28 report (Prevention of Future Death report).**
- **What to do if criticised by the coroner.**

4.2 Post inquest
When the coroner has heard all the evidence, they will 'sum up' and deliver the conclusion. This can be in 'short form' or a narrative verdict. It is important for you to obtain the outcome of the inquest, and if you are legally represented, the lawyer will make sure that this is received.

However, in some instances, a doctor can give evidence without receiving the final outcome, thus never knowing about potential criticism or if a 'Regulation 28' report will be issued. If you are without legal representation, it is important to find out the outcome by contacting the coroner's office directly.

When recording the cause of death, the coroner or jury may use one of the following terms:

- accident or misadventure

- alcohol/drug related
- industrial disease
- lawful/unlawful killing
- natural causes
- open (used when there is insufficient evidence for any other outcome)
- road traffic collision
- stillbirth
- suicide

Alternatively, or in addition, the coroner or jury may make a brief 'narrative' conclusion setting out the facts surrounding the death in more detail and explaining the reasons for the decision.

4.3 Regulation 28 reports – Prevention of Future Death Reports

Following the inquest, the coroner has a duty to make recommendations in cases where the evidence suggests that further avoidable deaths could occur and that, in the coroner's opinion, preventative action should be taken. The coroner will prepare a report, which will be sent to the person or authority that may have the power to take the appropriate steps to reduce the risk; they have a duty to provide a response within 56 days of the date the report is sent. If a doctor is concerned that they may be (or have been) criticised in the context of a coroner's inquest, including in a Regulation 28 Report, then they should contact their MDO at the earliest opportunity to seek advice about the appropriate steps to take. The coroner must send the report and response to the Chief Coroner. The Chief Coroner issues a summary of these reports, which is published on the Judiciary website at https://www.judiciary.uk/?s=Prevention+of+future+death+

Example of an issue raised in a Prevention of Future Death Report https://www.judiciary.uk/prevention-of-future-death-reports/chloe-burgess-prevention-of-future-deaths-report/

The MATTERS OF CONCERN are as follows –

The inquest heard evidence that the potential dangers of the combination of amitriptyline, paroxetine and ivabradine is not widely appreciated and does not trigger an alert on the prescribing software used in primary care or by pharmacists. The potential dangers related to a failure to metabolise amitriptyline which can, incrementally, lead to toxicity.

I am also concerned that those prescribing ivabradine should have a full understanding of the potential interaction with amitriptyline and paroxetine.

A useful resource for organizations to be able to identify national or organizational trends and disseminate lessons is the Preventable Deaths Tracker:

https://preventabledeathstracker.net

4.4 Civil proceedings
In an inquest, a determination may not be framed in such a way as to appear to determine any question of criminal or civil liability on the part of a named person (CJA 2009). However, when all the facts about the cause of death are known it is possible that civil proceedings may be brought and a claim for damages made after the inquest. Advice should be sought about the time limits and procedures that apply.

4.5 What to do if you are criticised by the coroner
On occasion, a doctor may be criticised in the coroner's report, or during an inquest. If this occurs, they are required to inform the GMC without delay. This may result in an investigation by the GMC. This is a rare occurrence but if it does happen, your MDO can guide you through the process and direct targeted CPD around the issues raised to demonstrate evidence of remediation to the GMC, reducing the risk of sanction.

Good medical practice, paragraph 75:

You must tell us without delay if, anywhere in the world:

a. you have accepted a caution from the police or ***been criticised by an official inquiry***

b. you have been charged with or found guilty of a criminal offence

c. another professional body has made a finding against your registration as a result of fitness to practise procedures.

4.6 Where to turn for help and support during an inquest
Any involvement in an inquest can be incredibly stressful, prompting mixed feelings and potentially concerns about your own professional welfare. These feelings can be managed with careful preparation and support. Seek support from colleagues who have experienced inquests in the past, as they will be in a good position to empathise with any concerns you may have but also give relevant information, regarding the process.
If you experience feelings of being overwhelmed such as anxiety, alert your GP and do not be afraid of seeking psychological support, which may aid your ability to negotiate the inquest.

Finally, remember to seek support from your MDO. As soon as you are aware of an inquest contact your MDO.

Section Recap

1. **When the coroner has heard all of the evidence, they will deliver a conclusion – this can be short form such as suicide or a brief narrative conclusion. TRUE**

2. **The coroner has a duty to make recommendations in cases where the evidence suggests that further avoidable deaths could occur, this is called a Prevention of Future Death report. TRUE**

3. **If you are criticised by the coroner, you should ignore this. FALSE**

   If you are criticised by the coroner, it is important that you discuss this with your MDO, to consider whether self-referral to the GMC is required and what remediation, on the doctor's part, would be useful.

Section Summary

In this section, we covered the following:

- **What happens after the inquest.**
- **What is a regulation 28 report.**
- **What to do if criticised by the coroner**

1. **If the evidence suggests that further avoidable deaths could occur the coroner will produce a regulation 28 report.**
2. **A response to a regulation 28 report needs to be supplied within 56 days.**
3. **Civil proceedings may follow an inquest.**
4. **A doctor may need to self-refer to the GMC if he or she is criticised.**